

Dr B White

Neurosurgon

City Hospital

Newtown

05/07/2014

Dear Dr White,

Re: Mr George Poulos

I am writing to refer Mr Poulos, a 45-year-old stockbroker, whose symptoms and signs are suggestive of low back pain, probably discogenic, with radiculopathy. Your expert assessment and further management would be highly appreciated.

On 21/6/2014, Mr Poulos attended my clinic complaining of a 2-day history of sudden severe low back pain, after lifting heavy objects, associated with back stiffness and inability to walk. His examination revealed loss of lumbar lordosis. However, no scoliosis was detected. Pain was experienced on flexion movement of spine, and was severe to limit the extension movement. There were intact sensation and both patellar and ankle reflexes. His straight leg raise test was 90 at right and left. Consequently, analgesic was prescribed, and he was advised to get some rest. A review after 1 week was arranged.

Over the subsequent visits, Mr Poulos condition has deteriorated. He has developed progressive pain, with downward extension, coincided with a difficulty in movement and tingling sensation in the right calf. Lumbar flexion has been limited, and other movements have become restricted. The straight leg raise test has shown deterioration. There has been loss of touch sensation over the distal calf and plantar aspect of foot. In addition, ankle reflex has been lost. Kindly note that Mr Poulos is allergic to Pethidine, penicillins and radiographic contrast agent. He, who is smoker and alcohol consumer, has a past history of head injury.

In view of ^{the} above, kindly arrange for a magnetic resonance imaging if you find it beneficial, and counsel Mr Poulos, regarding the possibility of surgery.

Should you have any queries, please do not hesitate to contact me.

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Yours sincerely,
Doctor

[CANDIDATE NO.] WRITING SUB-TEST - ANSWER BOOKLET 03/04

